5.1 Stroke (Ischemic and Hemorrhagic)

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family say his right side went weak and his speech slurred 2 hours ago **Objective** • Weak on the right, muscle strength 2/5 • Face droops on one side • Cannot turn in bed by himself	**ACTUAL** **Impaired physical mobility** related to neuromuscular impairment as evidenced by hemiparesis and inability to turn independently.	Within 1 week, joint ROM kept without contractures and he has started moving with help.	• Check neurological status and GCS as ordered (often every hour in the acute phase). • Keep the affected limbs in neutral alignment and support the weak shoulder. • Turn him every 2 hours. • Passive ROM 3 times a day. • Start rehabilitation early with the physiotherapist.	No contractures, movement getting better: **goal met**.
Subjective • Tries to talk but the words do not come out clearly **Objective** • Expressive aphasia • Frustrated	**ACTUAL** **Impaired verbal communication** related to cerebral injury as evidenced by inability to form words.	Within 3 days he can make his basic needs known by other means.	• Speak slowly, in short simple sentences. • Give him time to answer and do not finish his sentences for him. • Use picture boards and gestures. • Refer to the speech therapist.	Gets his basic needs across: **goal met**.
Subjective • Coughs when he drinks **Objective** • Face weak • Gag reflex reduced	**RISK** **Risk for aspiration** as evidenced by risk factors: impaired swallowing and reduced level of consciousness.	No aspiration.	• Nothing by mouth until the swallow screen is done. • Sit him upright for meals and for 30 minutes afterwards. • Give thickened fluids as recommended. • Keep suction available.	Breath sounds clear: **goal met**.